

Geriatric Physical Therapy Assessment Booklet

Outcome Measures for Initial Evaluation & Follow-up

Patient Information			
Name	_____	Sex	☐ M ☐ F
Age	_____	Diagnosis	_____
Assessment Date	_____	Therapist	_____

Assessment type: ☐ Initial Evaluation ☐ Follow-up (Session ____)

This booklet compiles internationally validated, age-appropriate subjective and objective outcome measures for easy clinical use and progress tracking. Each measure includes scoring instructions and clinical interpretation.

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1. Patient History

Chief Complaint

Onset: _____ Location: _____ Quality: ☐ dull ☐ sharp ☐ aching ☐ numbness ☐ other _____

Duration: ☐ constant ☐ intermittent Aggravating: _____ Relieving: _____

History of Present Illness

Past Medical History (tick all that apply)

- | | |
|---|--|
| ☐ Hypertension | ☐ Diabetes |
| ☐ Coronary heart disease | ☐ Arrhythmia |
| ☐ Osteoporosis | ☐ Stroke |
| ☐ Parkinson's disease | ☐ Arthritis |
| ☐ Chronic lung disease | ☐ Vision/hearing impairment |
| ☐ Cognitive impairment | ☐ Urinary condition |
| ☐ Other: _____ | |

Fall History (past 12 months)

No. of falls	☐ 0 ☐ 1 ☐ ≥2 (____ times)	Injury	☐ none ☐ soft tissue ☐ fracture ☐ head injury
Fear of falling	☐ none ☐ mild ☐ moderate ☐ severe (limits activity)	Getting up	☐ independent ☐ needs assistance

Medications & Surgical History

Long-term meds: ☐ anticoagulant ☐ antihypertensive ☐ antidiabetic ☐ sedative/hypnotic ☐ analgesic ☐ other: _____

Surgical history: _____

Social & Functional Status

Living situation	☐ alone ☐ with family ☐ facility	Assistive device	☐ none ☐ cane ☐ walker ☐ wheelchair
Exercise habit	☐ none ☐ occasional ☐ regular (____ × /week)	Smoking / Alcohol	☐ none ☐ smoking ☐ alcohol

Prior rehab	☐ none ☐ yes: _____	Patient expectations	_____
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Allergies & Family History

Allergies: ☐ none ☐ drug (_____) ☐ food/other (_____)

Family history: ☐ unremarkable ☐ yes: _____ (e.g. osteoporosis, cardiovascular, neurodegenerative)

Red Flag Screening (any positive — proceed with caution / refer)

- | | |
|---|---|
| ☐ Night pain / pain at rest | ☐ Unexplained weight loss > 5 kg |
| ☐ Recent trauma / fall | ☐ Persistent fever / night sweats |
| ☐ Bowel / bladder change | ☐ History of malignancy |
| ☐ Progressive weakness / atrophy | ☐ Coagulation disorder / anticoagulant |

Neurological Symptoms

Numbness/tingling	☐ none ☐ upper limb ☐ lower limb ☐ other_____	Weakness	☐ none ☐ upper limb ☐ lower limb
Dizziness/vertigo	☐ none ☐ occasional ☐ frequent ☐ postural	Vision change	☐ none ☐ diplopia ☐ blurred
Speech/swallow	☐ none ☐ yes: _____	Gait abnormality	☐ none ☐ shuffling ☐ wide-based

ADL Baseline

Task	Independent	Partial assist	Dependent
Eating	☐	☐	☐
Bathing	☐	☐	☐
Dressing	☐	☐	☐
Toileting	☐	☐	☐
Bed – chair transfer	☐	☐	☐
Level walking	☐	☐	☐
Stairs	☐	☐	☐
Shopping/cooking (IADL)	☐	☐	☐

Home Environment & Rehab Goal

Home hazards	☐ loose rugs/thresholds ☐ no grab bars ☐ poor lighting ☐ steep stairs ☐ none
SMART goal	_____

2. Numeric Pain Rating Scale (NPRS)

Rate pain on a 0 – 10 scale. 0 = no pain, 10 = worst pain imaginable.

0	1	2	3	4	5	6	7	8	9	10
No pain					Moderate					Worst

Time point	Pain score (0 – 10)
Now (at rest)	
Least in past 24 h	
Worst in past 24 h	
With activity	

Pain Location Diagram

Mark the pain site (shade = pain, X = worst point):

FRONT	BACK
(patient marks pain here)	(patient marks pain here)

MCID: a decrease ≥ 2 points is considered meaningful improvement.

3. EuroQol 5-Dimension 5-Level (EQ-5D-5L)

For each dimension, tick the ONE statement that best describes **your health today**:

Mobility

- ☐ I have no problems in walking about
- ☐ I have slight problems walking about
- ☐ I have moderate problems walking about
- ☐ I have severe problems walking about
- ☐ I am unable to walk about

Self-care

- ☐ I have no problems washing or dressing myself
- ☐ I have slight problems washing or dressing myself
- ☐ I have moderate problems washing or dressing myself
- ☐ I have severe problems washing or dressing myself
- ☐ I am unable to wash or dress myself

Usual activities

- ☐ I have no problems doing my usual activities
- ☐ I have slight problems doing my usual activities
- ☐ I have moderate problems doing my usual activities
- ☐ I have severe problems doing my usual activities
- ☐ I am unable to do my usual activities

Pain / discomfort

- ☐ I have no pain or discomfort
- ☐ I have slight pain or discomfort
- ☐ I have moderate pain or discomfort
- ☐ I have severe pain or discomfort
- ☐ I have extreme pain or discomfort

Anxiety / depression

- ☐ I am not anxious or depressed
- ☐ I am slightly anxious or depressed
- ☐ I am moderately anxious or depressed
- ☐ I am severely anxious or depressed
- ☐ I am extremely anxious or depressed

The 5 dimensions can be converted to a health utility index (0 – 1). MCID: utility ↑ 0.05 – 0.08.

4. Handgrip Strength

Instrument: Jamar or equivalent hydraulic dynamometer, handle at position 2.

Position: seated, shoulder adducted and neutral, elbow flexed 90° , forearm neutral, wrist $0 - 30^\circ$ extension.

Method: alternate left/right, 3 trials each, ≥ 30 s rest between. Encourage maximal effort.

	Trial 1 (kg)	Trial 2 (kg)	Trial 3 (kg)	Best (kg)
Left				
Right				
Both avg.	—	—	—	

Reference values (AWGS 2019)

Sex	Sarcopenia cutoff	Note
Male	< 26 kg	
Female	< 18 kg	

MCID: an increase $\geq 5 - 6$ kg is considered a meaningful improvement.

5. Short Physical Performance Battery (SPPB)

Three sub-tests; total score 0 – 12. ≤ 8 indicates frailty / high fall risk.

General Safety for Performance Tests (SPPB / TUG / BBS / 2MST)

- Therapist stands beside and slightly behind the patient throughout to guard against falls.
- Stop immediately if: chest pain/tightness, severe dyspnoea, dizziness/ataxia, pallor/diaphoresis, $\text{SpO}_2 < 85\%$, or the patient asks to stop.
- Before endurance testing (2MST): defer if resting SBP > 180 or DBP > 110 , or $\text{SpO}_2 < 90\%$.

A. Standing Balance

Requirement: stand without assistive device. Hold each position up to 10 s.

Position	Able?	Time (s)	Score
1. Feet side-by-side	☐ yes / ☐ no		☐ 0 / ☐ 1
2. Semi-tandem (heel of one foot beside big toe of the other)	☐ yes / ☐ no		☐ 0 / ☐ 1
3. Tandem (heel of one foot directly in front of toes of other)	☐ yes / ☐ no	10 s = 2 3 – 9.99 s = 1 < 3 s = 0 Record: ____ s	☐ 2 ☐ 1 ☐ 0

If position 1 failed → 0 and stop; if position 2 failed → 1, proceed to gait.

B. 4-Metre Gait Speed

Requirement: walk 4 m at usual pace (assistive device permitted); 2 trials, use the faster.

	Trial 1			Trial 2	
Walking time (s)					
Time	≤ 4.82 s	4.83 – 6.20 s	6.21 – 8.70 s	> 8.70 s	Unable
Score	4	3	2	1	0

Gait speed score: _____

C. Five Chair Stands

Requirement: arms folded across chest, rise fully from a standard chair (seat ~43 – 45 cm) and sit back down, 5 times, timed.

Screening trial: do 1 stand first to confirm independence.

Able?	☐ yes Time: _____ s				☐ no → 0
Time	≤ 11.19 s	11.20 – 13.69 s	13.70 – 16.69 s	16.70 – 59.99 s	> 60 s
Score	4	3	2	1	0

Chair stand score: _____

Sub-test	A. Balance (0 – 4)	B. Gait speed (0 – 4)	C. Chair stand (0 – 4)	SPPB total (0 – 12)
Score				

SPPB Interpretation

- 0 – 3: severe limitation, high fall risk • 4 – 6: moderate limitation
- 7 – 9: mild limitation • 10 – 12: good function
- MCID: ≥ 1.0 point

6. Timed Up and Go (TUG)

Procedure

1. Patient seated in a standard armchair (seat ~45 cm), back against the chair.
2. Place a marker on the floor 3 m ahead of the chair (tape or cone).
3. Instruction: “On the word ‘go’, stand up, walk to the 3 m marker at a safe and brisk pace, turn, walk back, and sit down again.”
4. Time from “go” to when the patient’s buttocks touch the seat again.
5. One practice trial, then 1 – 2 timed trials; record the best.
6. The patient may use their usual assistive device (e.g. cane).

	Time (s)	Assistive device?	Notes
Practice (unscored)		☐ yes / ☐ no	
Trial 1		☐ yes / ☐ no	
Trial 2 (optional)		☐ yes / ☐ no	
Best			

Interpretation

- < 10 s: normal, freely mobile
- 10 – 19 s: mostly independent; mild gait/balance impairment possible
- ≥ 13.5 s: increased fall risk
- ≥ 20 s: obvious mobility impairment — intervention needed
- ≥ 30 s: severe impairment; assistance needed for ADLs

MCID: a decrease $\geq 2 - 3$ s is considered a meaningful improvement.

7. Two-Minute Step Test (2MST) — Home-friendly

The 2MST is the recognised substitute for the 2MWT/6MWT when space is limited (e.g. at home); it assesses aerobic and lower-limb endurance. From the Senior Fitness Test.

Setup & equipment

- Only a standing spot is needed (beside a wall or sturdy chair); no long corridor.
- Stopwatch; wall tape or a resistance band (to mark knee height).
- Sphygmomanometer, pulse oximeter, Borg scale (RPE 6 – 20).
- A sturdy chair / walker for the patient to lightly touch for balance (balance only, not for lifting).

Knee-height mark

Mark the midpoint between the upper border of the patella and the iliac crest with wall tape, or a band between two chairs. Each step counts only if the knee reaches this height.

Pre-test assessment

Measure	Value	Note
Resting HR	_____ bpm	normal 60 – 100
Blood pressure	_____ / _____ mmHg	defer if resting SBP > 180
SpO ₂	_____ %	caution if < 90%
Borg RPE (rest)	_____ (6 – 20)	

Instructions

“March in place for 2 minutes, lifting each knee up to the mark. Aim to complete as many correct steps as possible. You may slow down or pause, but the timer keeps running. Ready? Go!”

- **Counting:** each knee lift reaching the mark counts as 1 step (usually count one side, e.g. right knee); record the 2-min total.
- Prompt time every 30 s with standard encouragement; at 2 min → “Stop!” Record total steps.
- The therapist must stand behind and to the side to guard against falls.

Result

Total steps in 2 min	_____ steps	Knee-height mark: _____ cm
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Post-test assessment

Measure	Value	Note
HR	_____ bpm	

Measure	Value	Note
Blood pressure	_____ / _____ mmHg	
SpO ₂	_____ %	concern if < 88% or drop > 4%
Borg RPE (post)	_____ (6 – 20)	
Borg dyspnoea	_____ (0 – 10)	modified Borg dyspnoea scale

Age- & sex-specific reference (~50th percentile, steps)

Age	Male (steps)	Female (steps)
60 – 69	85 – 95	75 – 87
70 – 79	75 – 87	65 – 80
80 – 89	60 – 76	55 – 68

- **Markedly below the reference** indicates reduced aerobic capacity and higher fall/disability risk.
- **MCID:** not firmly established; ~ ≥ 10 steps (or ~10% of baseline) is suggested as a reference threshold.

8-A. Oswestry Disability Index (ODI) — Low Back Pain

For each section, tick the ONE statement that best describes **your status today**.

1. Pain intensity

- ☐ (0) I have no pain at the moment.
- ☐ (1) The pain is very mild at the moment.
- ☐ (2) The pain is moderate at the moment.
- ☐ (3) The pain is fairly severe at the moment.
- ☐ (4) The pain is very severe at the moment.
- ☐ (5) The pain is the worst imaginable at the moment.

2. Personal care (washing, dressing, etc.)

- ☐ (0) I can look after myself normally without causing extra pain.
- ☐ (1) I can look after myself normally but it causes extra pain.
- ☐ (2) It is painful to look after myself and I am slow and careful.
- ☐ (3) I need some help but manage most of my personal care.
- ☐ (4) I need help every day in most aspects of self-care.
- ☐ (5) I do not get dressed; I wash with difficulty and stay in bed.

3. Lifting

- ☐ (0) I can lift heavy weights without extra pain.
- ☐ (1) I can lift heavy weights but it gives extra pain.
- ☐ (2) Pain prevents me lifting heavy weights off the floor, but I can manage if conveniently placed (e.g. on a table).
- ☐ (3) Pain prevents me lifting heavy weights, but I can manage light-to-medium weights if conveniently placed.
- ☐ (4) I can lift very light weights.
- ☐ (5) I cannot lift or carry anything.

4. Walking

- ☐ (0) Pain does not prevent me walking any distance.
- ☐ (1) Pain prevents me walking more than one mile (~1.6 km).
- ☐ (2) Pain prevents me walking more than ½ mile (~800 m).
- ☐ (3) Pain prevents me walking more than ¼ mile (~400 m).
- ☐ (4) I can only walk using a stick or crutches.
- ☐ (5) I am in bed most of the time.

5. Sitting

- ☐ (0) I can sit in any chair as long as I like.
- ☐ (1) I can only sit in my favourite chair as long as I like.
- ☐ (2) Pain prevents me sitting more than one hour.
- ☐ (3) Pain prevents me sitting more than ½ hour.
- ☐ (4) Pain prevents me sitting more than 10 minutes.

- ☐ (5) Pain prevents me sitting at all.

6. Standing

- ☐ (0) I can stand as long as I want without extra pain.
- ☐ (1) I can stand as long as I want but it gives extra pain.
- ☐ (2) Pain prevents me standing more than 1 hour.
- ☐ (3) Pain prevents me standing more than ½ hour.
- ☐ (4) Pain prevents me standing more than 10 minutes.
- ☐ (5) Pain prevents me standing at all.

7. Sleeping

- ☐ (0) My sleep is never disturbed by pain.
- ☐ (1) My sleep is occasionally disturbed by pain.
- ☐ (2) Because of pain I have less than 6 hours' sleep.
- ☐ (3) Because of pain I have less than 4 hours' sleep.
- ☐ (4) Because of pain I have less than 2 hours' sleep.
- ☐ (5) Pain prevents me from sleeping at all.

8. Sex life (if applicable)

- ☐ (0) My sex life is normal and causes no extra pain.
- ☐ (1) My sex life is normal but causes some extra pain.
- ☐ (2) My sex life is nearly normal but is very painful.
- ☐ (3) My sex life is severely restricted by pain.
- ☐ (4) My sex life is nearly absent because of pain.
- ☐ (5) Pain prevents any sex life at all.

9. Social life

- ☐ (0) My social life is normal and causes no extra pain.
- ☐ (1) My social life is normal but increases the degree of pain.
- ☐ (2) Pain has no significant effect on my social life apart from limiting more energetic interests (e.g. sport).
- ☐ (3) Pain has restricted my social life and I do not go out as often.
- ☐ (4) Pain has restricted my social life to my home.
- ☐ (5) I have no social life because of pain.

10. Travelling

- ☐ (0) I can travel anywhere without pain.
- ☐ (1) I can travel anywhere but it gives extra pain.
- ☐ (2) Pain is bad but I manage journeys over two hours.
- ☐ (3) Pain restricts me to journeys of less than one hour.
- ☐ (4) Pain restricts me to short necessary journeys under 30 minutes.
- ☐ (5) Pain prevents me travelling except to receive treatment.

ODI Scoring

Total: _____ / 50

ODI% = (Total ÷ 50) × 100 = _____ %

(If the sex-life item is not applicable, divide by 45 instead.)

Interpretation: 0 – 20% minimal | 21 – 40% moderate | 41 – 60% severe | 61 – 80% crippling | 81 – 100% bed-bound

MCID: a decrease $\geq$ 10 percentage points is considered a meaningful improvement.

8-B. QuickDASH Upper-Limb Outcome

Answer based on the **past week**; rate overall function even if the two arms differ.

Difficulty (1 = no difficulty, 5 = unable)

Item	1 No diff.	2 Slight	3 Moderate	4 Severe	5 Unable
1. Open a tight or new jar	☐	☐	☐	☐	☐
2. Do heavy household chores (e.g. wash walls, floors)	☐	☐	☐	☐	☐
3. Carry a shopping bag or briefcase	☐	☐	☐	☐	☐
4. Wash your back	☐	☐	☐	☐	☐
5. Use a knife to cut food	☐	☐	☐	☐	☐
6. Recreational activities taking force/impact through the arm (e.g. racquet sports, hammering)	☐	☐	☐	☐	☐

Symptom severity (past week)

Item	1 None	2 Slight	3 Moderate	4 Severe	5 Extreme
7. Pain in your arm, shoulder or hand	☐	☐	☐	☐	☐
8. Pain in your arm, shoulder or hand during a specific activity	☐	☐	☐	☐	☐
9. Tingling (pins and needles) in your arm, shoulder or hand	☐	☐	☐	☐	☐

Impact (past week)

10. Does your arm/shoulder/hand problem interfere with your work or daily activities?	☐ not at all ☐ slightly ☐ moderately ☐ severely ☐ extremely
11. Does your arm/shoulder/hand problem interfere with your sleep?	☐ not at all ☐ slightly ☐ moderately ☐ severely ☐ extremely

Scoring

QuickDASH = [(sum of item scores ÷ number of items) – 1] × 25 = _____

(At least 10/11 items must be completed.) Range 0 – 100; higher = more disability.

MCID: a decrease $\geq 10 - 15$ points is considered a meaningful improvement.

8-C. Activities-specific Balance Confidence (ABC) Scale

Rate your confidence that you **will not lose your balance** while performing each activity.

Choose from 0% (no confidence) to 100% (completely confident).

#	Activity	Confidence (0 – 100%)
1	Walk around the house	
2	Walk up or down stairs	
3	Bend over and pick up a slipper from the floor	
4	Reach for a small can off a shelf at eye level	
5	Stand on tiptoes and reach for something above your head	
6	Stand on a chair and reach for something	
7	Sweep the floor	
8	Walk out the front door to a car parked in the driveway	
9	Get into or out of a car	
10	Walk across a parking lot to the mall	
11	Walk up or down a ramp	
12	Walk through a crowded mall where people walk past quickly	
13	Are bumped into by people as you walk through the mall	
14	Step onto or off an escalator while holding the railing	
15	Step onto or off an escalator while holding parcels (cannot hold the railing)	
16	Walk outside on icy sidewalks	

Scoring

ABC score = mean of all item ratings = _____ %

• < 67%: increased fall risk • 50 – 80%: moderate function • > 80%: high function

MCID: an increase ≥ 10 percentage points is considered a meaningful improvement.

8-D. Berg Balance Scale (BBS)

BBS is the gold-standard balance measure for older adults: 14 items, each scored 0 – 4, total 0 – 56.

Instructions: ask the patient to perform each task; score by quality of performance. Guard the patient.

#	Task	0	1	2	3	4	Score
1	Sitting to standing	☐	☐	☐	☐	☐	
2	Standing to sitting	☐	☐	☐	☐	☐	
3	Transfers	☐	☐	☐	☐	☐	
4	Standing unsupported	☐	☐	☐	☐	☐	
5	Sitting unsupported	☐	☐	☐	☐	☐	
6	Standing with eyes closed	☐	☐	☐	☐	☐	
7	Standing with feet together	☐	☐	☐	☐	☐	
8	Reaching forward with outstretched arm	☐	☐	☐	☐	☐	
9	Retrieving object from floor	☐	☐	☐	☐	☐	
10	Turning to look behind	☐	☐	☐	☐	☐	
11	Turning 360 degrees	☐	☐	☐	☐	☐	
12	Placing alternate foot on stool	☐	☐	☐	☐	☐	
13	Standing with one foot in front	☐	☐	☐	☐	☐	
14	Standing on one leg	☐	☐	☐	☐	☐	

Scoring: 0 = unable | 1 = needs much help | 2 = needs little help | 3 = independent but unsteady | 4 = normal, independent

BBS Total _____ / 56

Interpretation

- 0 – 20: poor balance, high fall risk (often wheelchair)
- 21 – 40: acceptable balance; needs assistive device; moderate fall risk
- 41 – 56: good balance; independent gait; low fall risk
- **Fall-risk cutoff:** < 45 indicates increased fall risk (some sources use < 50)

MCID: an increase ≥ 4 points (general elderly) / ≥ 6 points (post-stroke) is considered meaningful.

Tip: the BBS takes ~15 – 20 min. If time is limited, use the SPPB balance sub-test + TUG for quick screening.

9. Assessment Summary

Core Outcome Measure Results

Measure	Initial Date: ____	Follow-up Date: ____	Change	Met MCID?
NPRS (pain 0 – 10)				☐ Y / ☐ N
EQ-5D-5L (utility)				☐ Y / ☐ N
Handgrip (best, kg)				☐ Y / ☐ N
SPPB (total 0 – 12)				☐ Y / ☐ N
TUG (s)				☐ Y / ☐ N
2MST (steps)				☐ Y / ☐ N
BBS (total 0 – 56)				☐ Y / ☐ N
Regional test: _____				☐ Y / ☐ N
Regional test: _____				☐ Y / ☐ N

Fall Risk Stratification

Tick any positive item; grade by the number of positives.

☐ SPPB $\leq$ 8	☐ TUG $\geq$ 13.5 s
☐ Gait speed < 1.0 m/s	☐ ABC < 67%
☐ BBS < 45	☐ $\geq$ 2 falls in past 12 months

Overall fall-risk level: ☐ Low (0) ☐ Moderate (1 – 2) ☐ High ($\geq$ 3)

Suggested interventions: ☐ balance training ☐ lower-limb strengthening ☐ assistive-device review ☐
home-modification referral ☐ medication review (to MD) ☐ fall-prevention education

Clinical Impression

Treatment Goals

1. _____
2. _____
3. _____

Plan

☐ Outpatient PT Frequency: _____ $\times$ /week Expected duration: _____ weeks

☐ Home exercise programme ☐ Group class ☐ Community rehab referral ☐ Other: _____

Therapist signature: _____ Date: _____

Appendix 1 — MCID & Cutoff Reference

Measure	Range	MCID	Cutoff / high-risk threshold
NPRS (pain)	0 – 10	↓ ≥ 2 points	—
EQ-5D-5L (utility)	0 – 1	↑ 0.05 – 0.08	—
Handgrip (male)	kg	↑ 5 – 6 kg	< 26 kg sarcopenia risk
Handgrip (female)	kg	↑ 5 – 6 kg	< 18 kg sarcopenia risk
SPPB	0 – 12	↑ ≥ 1.0 point	≤ 8 frailty / high fall risk
TUG	seconds	↓ 2 – 3 s	≥ 13.5 s increased fall risk
2MST (2-min step)	steps	↑ ≥ 10 steps	well below age norm → reduced capacity
BBS (Berg balance)	0 – 56	↑ ≥ 4 points	< 45 increased fall risk
ODI (low back)	0 – 100%	↓ ≥ 10%	> 40% severe disability
QuickDASH (upper limb)	0 – 100	↓ 10 – 15 points	—
ABC Scale (balance confidence)	0 – 100%	↑ 10%	< 67% fall risk

Appendix 2 — Borg Rating of Perceived Exertion (RPE 6 – 20)

Score	Description
6	No exertion at all (resting)
8	Extremely light
9	Very light (easy conversation, e.g. slow walk)
11	Light
13	Somewhat hard (talking takes effort)
15	Hard
17	Very hard (breathless; only brief phrases)
19	Extremely hard (near maximum)
20	Maximal exertion (exhaustion)

Appendix 3 — Exercise Prescription (FITT-VP)

Type	Intensity (RPE)	Frequency (×/wk)	Time (min)	Sets × reps	Notes
Aerobic (e.g. walking, cycle)					
Resistance (lower limb)					
Resistance (upper limb)					
Balance					

Type	Intensity (RPE)	Frequency (×/wk)	Time (min)	Sets × reps	Notes
Flexibility (stretching)					

Home Exercise Library

26 curated, senior-safe home exercises in five categories. Each entry lists the target, dosage, progression, and regression for individualised prescription based on assessment findings.

Usage notes

Warm-up: 5 – 10 min low-intensity (marching, joint mobility). Cool-down: stretch after session.

Caution: always stay near support for balance exercises. Stop immediately if pain, dizziness, or palpitations occur.

Resistance — Lower Limb

#	Target	Key cues	Dose	Progress ↑ / Regress ↓
1	Sit-to-Stand Quadriceps, glutes	Seated in a sturdy chair, feet shoulder-width. Lean forward, stand fully, sit slowly.	10 – 15 × 2 – 3	↑ Hold light dumbbell ↓ Use armrests
2	Glute Bridge Glutes, hamstrings	Supine, knees bent, feet flat. Squeeze glutes, lift hips, lower slowly.	10 × 2	↑ Single-leg bridge ↓ Smaller range
3	Side-lying Hip Abduction Gluteus medius	Side-lying, bottom knee bent. Lift top leg to 30 – 45°, lower slowly.	10 – 12 / side × 2	↑ Ankle weight ↓ Smaller range
4	Standing Calf Raise Gastrocnemius, soleus	Stand behind chair, light fingertip support. Rise onto toes, hold 2 s, lower.	10 – 15 × 2	↑ Single leg ↓ Seated calf raise
5	Wall Sit Quadriceps	Back against wall, feet 30 cm out. Slide down to semi-squat, hold, stand up.	Hold 10 – 30 s × 3	↑ Deeper squat ↓ Shallower angle
6	Step-up Quadriceps, glutes	In front of a 10 – 20 cm step. Step up, full stand, step down slowly.	8 – 10 / leg × 2	↑ Higher step ↓ Lower step

Resistance — Upper Limb

#	Target	Key cues	Dose	Progress ↑ / Regress ↓
7	Wall Push-up Pectorals, triceps, deltoids	Arm-length from wall, palms at shoulder height. Bend elbows, bring chest to wall, push back.	10 – 15 × 2	↑ Feet further back ↓ Closer to wall
8	Seated Band Row Rhomboids, latissimus	Seated, band around feet, elbows in. Pull band to waist, squeeze shoulder blades.	10 – 12 × 2	↑ Stiffer band ↓ Lighter band
9	Shoulder Press (Band) Deltoids, triceps	Seated or standing, band under feet. Press overhead, lower slowly to shoulders.	8 – 10 × 2	↑ Stiffer band ↓ Smaller range
10	Band Pull-apart Rear deltoids, rhomboids	Stand, band at chest height, arms forward. Pull band apart to sides, return slowly.	10 × 2	↑ Wider grip ↓ Narrower grip
11	Chair Triceps Dip Triceps	Seated at chair edge, hands gripping seat edge. Slide hips forward, bend elbows 5 – 10 cm, push up.	8 – 10 × 2	↑ Feet further out ↓ Smaller dip

Balance

#	Target	Key cues	Dose	Progress ↑ / Regress ↓
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12	Single-Leg Stand Static balance	Stand behind a sturdy chair. Lift one foot, hold, lower. Alternate legs.	Hold 10 – 30 s / leg × 3	↑ Eyes closed / no support ↓ Two-hand support
13	Tandem Stance Static balance	Stand heel-to-toe, near wall. Hold the tandem position, switch lead foot.	Hold 20 – 30 s × 3	↑ Eyes closed ↓ Semi-tandem
14	Heel-to-Toe Walk Dynamic balance	Along a wall or hallway. Walk placing heel to toe in a line.	10 steps × 3	↑ Backward walk ↓ Wider base
15	Weight Shift Dynamic balance	Stand, feet shoulder-width, near support. Shift weight side to side smoothly.	10 / side × 2	↑ Wider stance ↓ Hold support
16	Clock Reach Dynamic balance	Imagine a clock face on the floor. Reach one foot to 12, 3, 6, 9 positions.	8 / side × 2	↑ Further reach ↓ Shorter reach
17	Side Step Dynamic balance	Stand, feet hip-width, near wall. Step sideways 10 steps, return.	10 / side × 2	↑ Resistance band at ankles ↓ Shorter steps

Flexibility

#	Target	Key cues	Dose	Progress ↑ / Regress ↓
18	Calf Stretch Gastrocnemius, soleus	Stand facing wall, one foot back, straight knee. Lean forward, feel stretch in back calf.	Hold 20 – 30 s × 2 / leg	↑ Deeper lean ↓ Less lean
19	Hamstring Stretch Hamstrings	Seated, one leg extended. Hinge forward from hips, reach for toes.	Hold 20 – 30 s × 2 / leg	↑ Straighten knee more ↓ Slight knee bend
20	Chest / Pec Stretch Pectorals	Stand in a doorway, forearm on frame. Gently turn away, feel stretch across chest.	Hold 20 – 30 s × 2 / side	↑ Higher arm on frame ↓ Lower arm position
21	Hip Flexor Stretch Iliopsoas	Standing lunge, back knee straight. Tuck pelvis, shift weight forward gently.	Hold 20 – 30 s × 2 / leg	↑ Deeper lunge ↓ Shorter stance
22	Neck Range of Motion Cervical muscles	Seated, shoulders relaxed. Slow rotations, side bends, chin tucks.	5 reps each × 1 – 2	↑ Full pain-free range ↓ Smaller range

Aerobic

#	Target	Key cues	Dose	Progress ↑ / Regress ↓
23	Marching in Place Cardiovascular	Stand or sit, upright posture. Alternate lifting knees to hip height.	1 – 3 min × 2 – 3	↑ Higher knees / faster ↓ Seated march
24	Step-up Circuit Cardiovascular, legs	In front of a 10 – 15 cm step, rail nearby. Step up and down at steady rhythm.	1 – 3 min × 2	↑ Faster pace ↓ Lower step
25	Brisk Walk Cardiovascular	Flat, well-lit path; suitable footwear. Walk at a pace where talking is slightly effortful.	10 – 30 min	↑ Longer / faster ↓ Shorter duration
26	Seated Cycling Cardiovascular	Stationary bike, seat at hip-knee level. Cycle at moderate resistance, steady pace.	5 – 10 min	↑ Higher resistance ↓ Shorter duration

Appendix 5 — Suggested Assessment Flow (initial ~40 – 60 min)

#	Step	~Time
1	Informed consent & history (patient may complete questionnaires in waiting room)	5 min
2	Subjective: NPRS + EQ-5D-5L	5 min
3	SPPB (balance + 4 m gait + 5 chair stands)	8 min
4	Rest 2 – 3 min	3 min
5	TUG	3 min
6	Handgrip	3 min
7	2MST (home-friendly; with pre/post HR, BP, SpO ₂ , RPE)	5 min
8	Berg Balance Scale (if detailed balance assessment needed)	15 min
9	Optional regional tests (ODI / QuickDASH / ABC)	5 – 10 min
10	Summary & goal-setting	5 min
Total		~40 – 60 min